**جوستاف روسي انترناشيونال - مصر (EPE) ***

# طلب علاج أول مرة / متردد

**إسم المريض رباعياً :** جيهان فتوح عطاالله احمد
**المحافظة :** الشرقيه **الحي / المدينة :** منيا القمح **العنوان :** هندسه الرى
**رقم البطاقة :** 27501151301909 **رقم التليفون :** 01153283891 **تاريخ تسجيل الإستمارة :** 21-10-2026
**العمر(تاريخ الميلاد) :** 15-01-1975 **المهنة :** ربه منزل

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

**Name:** جيهان فتوح عطاالله احمد
**National ID:** 27501151301909

## تقرير طبي

**السيد**

**تحية طيبة وبعد,,**

| | |
|---|---|
| **إيماء للطلب المقدم من** | **بشأن تقرير طبي للمريض** |
| **بقسم:** | **رقم طبي:** |

**نتشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | |
|---|---|
| **تاريخ وساعة الدخول :** -- --:-- yyyy-mm-dd | **تاريخ وساعة الخروج :** PM 01:17 05-07-2026 |

**وبعد الكشف والفحوصات والمتابعة وجد انة**

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

| | |
|---|---|
| **التاريخ :** 05-07-2026 | **الوقت :** -- --:-- |
| | **الطبيب المعالج :** د/شيماء لاشين |


جمهورية مصر العربية
بطاقة تحقيق الشخصية
جيهان
فتوح عطاالله احمد
ش هندسة الري
مركز منيا القمح - الشرقية
٢٧٥٠١١٥١٣٠١٩٠٩
KA2121420


٢٧٥٠١١٥١٣٠١٩٠٩
٢٠٢٣/٠٦
حاصلة على دبلوم صنايع
أنثى
مسلمة
متزوجة
طارق سيد عثمان حسن
٨
البطاقة سارية حتى ٢٠٣٠/٠٦/٠٥

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

06/15/2026

# MDT FORM

| | | | |
|---|---|---|---|
| **Name:** | Jihan Fatouh Attallah El-Sayed | **MRN:** | 3894 |
| **Age:** | 51 Y | **ID:** | 27501151301909 |
| **Menopausal Sta** | | **Phone:** | 01153283891 |
| **NRS SCORE:** | | **Marital Status:** | |
| **BMI:** | | **Family History:** | |
| **CO Morbidity:** | | **Reason Of MDT:** | POST OP |

## Clinical Data:

**BX:**

**ER:** **PR:** **HER2:** **KI67:**

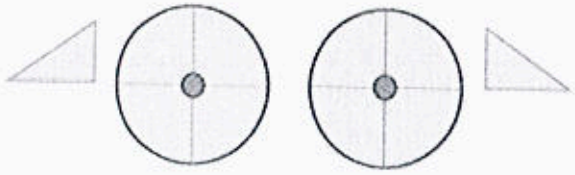
**Sonomammography:**

**Mets Workup:**

**History:**
NB,,,,PT DIDNT RECEIVE ANY( HP) PRE OP
JUST 6 CYCLES X TC RECEIVED

**Recommendations:**
post NACT, LT BCS AND SLNB , FREE LNS 0/9 , PPR, T1c N0

R L



TNM

**MDT Outcome & Plan**
adjuvant Herceptin
port
adjuvant hormonal according to E2 FSH LH

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|---|---|---|---|---|---|
| | | | | | |

| | |
|---|---|
| **Name:** جيهان فتوح عطاالله احمد | |
| **MR. :** 3894 | |

Detailed Medical Report

| | |
|---|---|
| **السيد** | |

**تحية طيبة وبعد,,**

| | | | |
|---|---|---|---|
| **إيماء للطلب المقدم من** | | **بشأن تقرير طبي للمريض** | |
| **بقسم:** | | **رقم طبي:** | |

**ننشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | | | |
|---|---|---|---|
| **تاريخ وساعة الدخول :** | | **تاريخ وساعة الخروج :** | |

**وبعد الكشف والفحوصات والمتابعة وجد انه**

**The patient is a 50-year-old with triple-positive breast cancer (TPBC) who received neoadjuvant chemotherapy (NACT) consisting of TCHP on cycle 4 with docetaxel 150 mg and carboplatin AUC 5. The patient has not yet received PHESGO. An echocardiogram performed on 18-Nov showed a left ventricular ejection fraction of 64 %. Imaging demonstrated an active tracer over the lumbar spine and MRI revealed disc abnormalities.**

**The patient continued neoadjuvant therapy for TPBC with TCHP on cycle 5, receiving docetaxel 150 mg and carboplatin AUC 5. Initiation of PHESGO was pending pending computed tomography results.**

**The patient underwent neoadjuvant chemotherapy (TC without HER2-targeted therapy) followed by breast-conserving surgery in May 2026 (ypT1c N0 M0). Multidisciplinary team decision was for adjuvant hormonal therapy (HT) and HER2-directed therapy according to an estrogen-receptor level of 7 % and an FSH level of 11.6 IU/L. The patient started Femara for the first month of hormonal therapy, with plans to begin HER2-directed therapy on 2 Sep 2026 for a duration of three months.**

**The patient is planned to receive adjuvant HER2-directed therapy with subcutaneous trastuzumab (c1) only. The most recent echocardiogram on 4 Aug showed an ejection fraction of 62 %.**

**تحريرا في ,**

| | | | |
|---|---|---|---|
| **التاريخ :** | 19-09-2026 | **الوقت :** | |

**الطبيب المعالج :** هنا اسامه عبد الرحيم عبد البر

**يعتمد ,**

GUSTAVE ROUSSY

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / جيهان فتوح عطا الله | Request No. | : 13002300 |
| Patient No. | : 27501151301909 | Request Date | : 28/09/2025 |
| Path No. | : SR-1363/25 | Verification Date | : 04/10/2025 |
| Sex | : Female | Financial Category | : جوستاف روسي - نفقة الدولة |
| Age | : 50 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : Mrs.Mayada Hegazy |

## Pathology Report

Clinical Data: Left breast mass with (HER2 neu) result elsewhere (Score 2, equivocal).
Nature of Specimen: Referred block (Tru-cut biopsy).

Gross Examination:
- One referred block labelled by (413B-2025).
Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; SISH.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* DCIS: Absent.
* Angioinvasion: Present.
* Perineural invasion: Absent.

* IPT (Positive control showed appropriate reactivity):
- SISH: Amplified
(Average Her2 copy No.: (4.15), Her2/CEP17 ratio: (2.17)).

Diagnosis:
Left breast mass, Referred block (Tru-cut biopsy), Requested (SISH):
- Invasive duct carcinoma, Grade 2.
IPT:
- SISH: Amplified, Group I.
(Average Her2 copy No.: (4.15), Her2/CEP17 ratio: (2.17)).

ISH groups:

Verified by :
Dr.Duaa Mohamed



CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY INTERNATIONAL

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / جيهان فتوح عطا الله | Request No. | : 13002300 |
| Patient No. | : 27501151301909 | Request Date | : 28/09/2025 |
| Path No. | : SR-1363/25 | Verification Date | : 04/10/2025 |
| Sex | : Female | Financial Category | : جوستاف روسي - نفقة الدولة |
| Age | : 50 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : Mrs.Mayada Hegazy |

## Pathology Report

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ebrahem Ghoneim.
Dr. Duaa Mohamed.

Verified by :
Dr.Duaa Mohamed



CAP ACCREDITED

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

| | |
|---|---|
| Name : jihan fatoh atiat allah | Date : 01-03-2026 12:33 |
| Branch : Portable | Medical No. : GRI-121957 |
| Ref.Physician Prof : ct chest | Ins.Company : |

## MULTISLICE NON CONTRAST CT SCAN OF THE CHEST:

**TECHNIQUE:** Multislice axial CT images of the chest with multiplanar reconstruction. No IV contrast was used.

**FINDINGS:**

In reviewing previous limited study report dated 10-6-2025, the current study showing

- Regressive course of the previously noted left breast mass lesion measuring about 24x15mm with internal hyperdensity in current study.
- Clear both lungs showing no sizable pulmonary nodules, consolidation or cavitation.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No plural or pericardial collection.
- No gross cardiomegaly
- Upper abdominal cuts are unremarkable.

*Dr. Nourhan Mohamed,MD*

**02/03/2026 02:58 PM**

*Dr. Nourhan Mohamed,MD*

MDT

| Patient Name | جيهان عطاالله احمد | Age | 50 |
|---|---|---|---|
| Sex | F | Special habits | |
| Family History | | Comorbidity | |
| Surgical History | | Telephone | 01153283891 |
| Menopausal Status | POST | Marital Status | Married |
| ID | 27501151301909 | Medical ID | 2739/2025 |
| Date | 28/10/2025 | Reason for MDT | NEW CASE |

## Medical Report

Affected breast: Left breast

MDT

Pathology: IDC Grade 2

ER: ⅞

PR: ⅞

HER2: +2 (SISH AMPLIFIED)

Ki67: 15%

T2 N0 M0

S.M: left breast solid mass lesion in LOQ 7.8x6.6 cm, 6 cm from nipple, irregular lobulated hypoechoic mass lesion 3.9x1.7 cm 4.8 mm from skin

Metsworkup:

Doctor's notes:

Recommendations: for upfront surgery left WLE + SLNB (OR) neoadjuvant as HER2 +ve

| MDT DECISION | For: clip insertion<br>NACT TCHP x 6 cycles<br>WLE + SLNB |
|---|---|

| Name | د/ خالد عبدالعزيز | د/ هاشم عبدالرحمن | د/ ابو بكر حجازي |
|---|---|---|---|
| Signature | | Dr. Hash<br>28/10/2025 | |

| | | | |
|---|---|---|---|
| ilent Name | : الاسكندرية / مروان فتوح عبد الله | | |
| ilent No. | : 27501151301909 | Request No. | : 13002300 |
| th No. | : SR-1363/25 | Request Date | : 28/09/2025 |
| )x | : Female | Verification Date | : 04/10/2025 |
| je | : 50 Y | Financial Category | : [illegible] |
| eferring Doc | : | Department | : Out Patient |
| | | Registered by | : Mrs.Mayada Hegazy |

## Pathology Report

Clinical Data: Left breast mass with (HER2 neu) result elsewhere (Score 2, equivocal).
Nature of Specimen: Referred block (Tru-cut biopsy).

Gross Examination:
One referred block labelled by (4138-2025).
Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; SISH.

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- DCIS: Absent.
- AngioInvasion: Present.
- Perineural Invasion: Absent.

IPT (Positive control showed appropriate reactivity):
SISH: Amplified.
Average Her2 copy No.: (4.15), Her2/CEP17 ratio: (2.17)).

Diagnosis:
Left breast mass, Referred block (Tru-cut biopsy), Requested (SISH):
Invasive duct carcinoma, Grade 2.
IPT:
SISH: Amplified, Group I.
Average Her2 copy No.: (4.15), Her2/CEP17 ratio: (2.17)).

SISH groups:

Verified by :
Dr.Duaa Mohamed



Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

| | | | |
|---|---|---|---|
| ıtient Name | : [illegible] | Request No. | : 13002300 |
| ıtient No. | : 27501151301909 | Request Date | : 28/09/2025 |
| th No. | : SR-1363/25 | Verification Date | : 04/10/2025 |
| ex | : Female | Financial Category | : [illegible] |
| ge | : 50 Y | Department | : Out Patient |
| eferring Doc | : | Registered by | : Mrs.Mayada Hegazy |

## Pathology Report

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Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is nore than 4 but less than 6. (Recounting give the same result).
Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is ıss than 4.

Ignature
r. Walaa Ebrahem Ghonelm.
r. Duaa Mohamed.

Verified by :
Dr.Duaa Mohamed



غرفة رقم (1) - الدور الأول - مبنى رقم (987) .. كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (1) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

NMTMRH

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

قسم الباثولوجي

# PATHOLOGY DEPARTMENT
# HISTOPATHOLOGICAL REPORT

| | |
|---|---|
| Patient name: مريض فتح مطلق احمد | Age: 50years Sex: female |
| Request type: ميلاد | Sub request type: |
| Hospital no: | Pathology no.: 4138-2025 |
| Department: صحة مرأة | Date of biopsy: 18/8/2025 |
| Referred by: دكتور حسن | Receiving date: 13/8/2025 |
| Nature of specimen: Tru-Cut Biopsy | I.D. Number:27501151301909 |
| Clinical diagnosis: Left breast mass | |

Gross pathology : Mulipite grayish white tissue cores up to 0.7cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER- PR- Her2- KI67

Microscopic picture: Section examined revealed core of tumor showing invasive duct carcinoma of no special type (NST). The tumor formed of anaplastic ductal cells infiltrating among desmoplastic stroma. Tumor cells showing marked anaplasia and moderate tendency to from tubules. There are moderate number of mitotic figures. Foci of lymphovascular invasion were detected.

Immunohistochemical Results:

ER: Moderate nuclear staining in 70% of tumor cells (Allred Score 7/8).

PR: Moderate nuclear staining of 70% of tumor cells (Allred Score 7/8).

Her2: Moderate-Incomplete membranous staining of >10% of tumor cells (Score 2+) For SISH.

KI67: Positive nuclear staining of 15% of tumor cell

Diagnosis: Left Breast Mass, Tru-Cut Biopsy:

Invasive Ductal Carcinoma, (NST) Grade II.
(ER 7/8, PR 7/8, Her2 Score 2+ For SISH, KI67 15%).

Signature:
Dr.Nermine Nabil.M.D.
Dr.Mona Mohamed. M.D

Pathology Lab. Manager

Nermine Nabil, M.D

NMTMRI

الهيئة العامة للمستلزمات والمعامل الطبية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

قسم الباثولوجي

# PATHOLOGY DEPARTMENT
# HISTOPATHOLOGICAL REPORT

| | |
|---|---|
| Patient name: [illegible] | Age: 50years Sex: female |
| Request type: [illegible] | Sub request type: |
| Hospital no: | Pathology no.: 4138-2025 |
| Department: [illegible] | Date of biopsy: 18/8/2025 |
| Referred by: [illegible] | Receiving date: 13/8/2025 |
| Nature of specimen: Tru-Cut Biopsy | I.D. Number:27501151301909 |
| Clinical diagnosis: Left breast mass | |

Gross pathology : Mulipite grayish white tissue cores up to 0.7cm long, totally submitted.

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Microscopic picture: Section examined revealed core of tumor showing invasive duct carcinoma of no special type (NST). The tumor formed of anaplastic ductal cells infiltrating among desmoplastic stroma. Tumor cells showing marked anaplasia and moderate tendency to from tubules. There are moderate number of mitotic figures. Foci of lymphovascular invasion were detected.

Immunohistochemical Result:

ER: Moderate nuclear staining in 70% of tumor cells (Allred Score 7/8).

PR: Moderate nuclear staining of 70% of tumor cells (Allred Score 7/8).

Her2: Moderate incomplete membranous staining of >10% of tumor cells (Score 2+) for SISH.

Ki67: Positive nuclear staining of 15% of tumor cell

Diagnosis: Left Breast Mass, Tru-Cut Biopsy:

Invasive Ductal Carcinoma, (NST) Grade II .
(ER 7/8, PR 7/8, Her2 Score 2+ For SISH, Ki67 15%).

Signature:
Dr.Nermine Nabil M.D
Dr.Mona Mohamed, M.D

Pathology Lab. Manager

Nermine Nabil, M.D

| | |
|---|---|
| Patient ID: 86568 | Patient Name (Eng): Gehan Fatoh Atalla |
| [illegible] | 2025/08/06 11:10:24 AM :Report Date |

## Women's Health Outreach Program

Name: Gehan Fatoh Atalla
ate: 6/8/2025

## Bilateral Full field Digital Mammography

Clinical summary: 50 y old patient coming left breast lump two weeks ago .

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR B).
- LOQ of the left breast is the seat of irregular spiculated hyper dense mass density with multiple long spicules occupying an area (mass and spicules ) measures 7.8x6.6 cm 6 cm from nipple .
- Ultrasound demonstrated 3-4 o'clock Zone 8 shows irregular lobulated spiculated hypoechoic solid mass lesion measures 3.9x1.7 cm ,4.8 mm from skin ,It shows increased vascularity by color Doppler study
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Right breast

- Scattered fibro glandular densities (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Left breast (BIRADS 4 c ): LOQ suspicious solid mass lesion (above described ) .

Right breast (BIRADS 1): Normal study.

Recommendations:

For left breast tissue biopsy and histopathological correlation .

Dr.Nashwa A. El Kader

JSTAVE/
OUSSY
INATIONAL

| | |
|---|---|
| lient Name: Jihan Fattouh Atalla Ahmed | Date : 06/10/2025 |
| am : CT CAP+C (CT Chest Abd. Pelvis) | Age : 60 Year(s) |
| idical ID: 2739/2026 | Pat. ID: 121957 |

MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

REVEALED:

## I- CHEST:

Known case of left breast cancer showed left breast mass lesion measuring 28 x 25 mm
No pulmonary nodules or masses.
No consolidation or collapse.
No hilar or mediastinal masses or lymph node enlargement.
No pleural or pericardial collection.
Normal plain CT appearance of the heart and great vessels.

## II- ABDOMEN AND PELVIS:

Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness. No stones or back pressure changes.
Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
The urinary bladder shows no evidence of gross masses or diverticulae or stones.
Normal appearance of the pelvic organs with no pelvic masses or collections.
No abdominal or pelvic lymphadenopathy.
No ascites.

## nion:

- Known case of left breast cancer.
- No hepatic or pulmonary metastatic lesion

*er Ahmed*

*er Ahmed*

*Nuclear Medicine Unit*

*وحدة الطب النووى*

*Jihan Fatouh Atallah* *Date:* ٢٣/١٢/٢٠٢٥

## Tc ٩٩m MDP SKELETAL SCINTIGRAPHY

**History**

- A ٥٠-year-old lady with history of breast cancer.

**Technique**

- Anterior and posterior whole-body scans were acquired ٢ hours post injection of ٢٠ mCi of the tracer.

**Findings**

- Active tracer uptake is seen at sternum and upper lumbar vertebrae.
- Features suggestive of marked degenerative changes at different levels of spine (especially upper dorsal vertebrae) and large joints, for follow-up.

**IMPRESSION**

- Abnormal equivocal tracer uptake pattern at upper lumbar spine and sternum as described calling for further clinical/regional radiological correlation and follow-up.

**Appreciated,**

**Dr.Hossam Sekkina, M.Sc.**

**Dr. Alaa Nagi, MD**
**Nuclear Medicine**

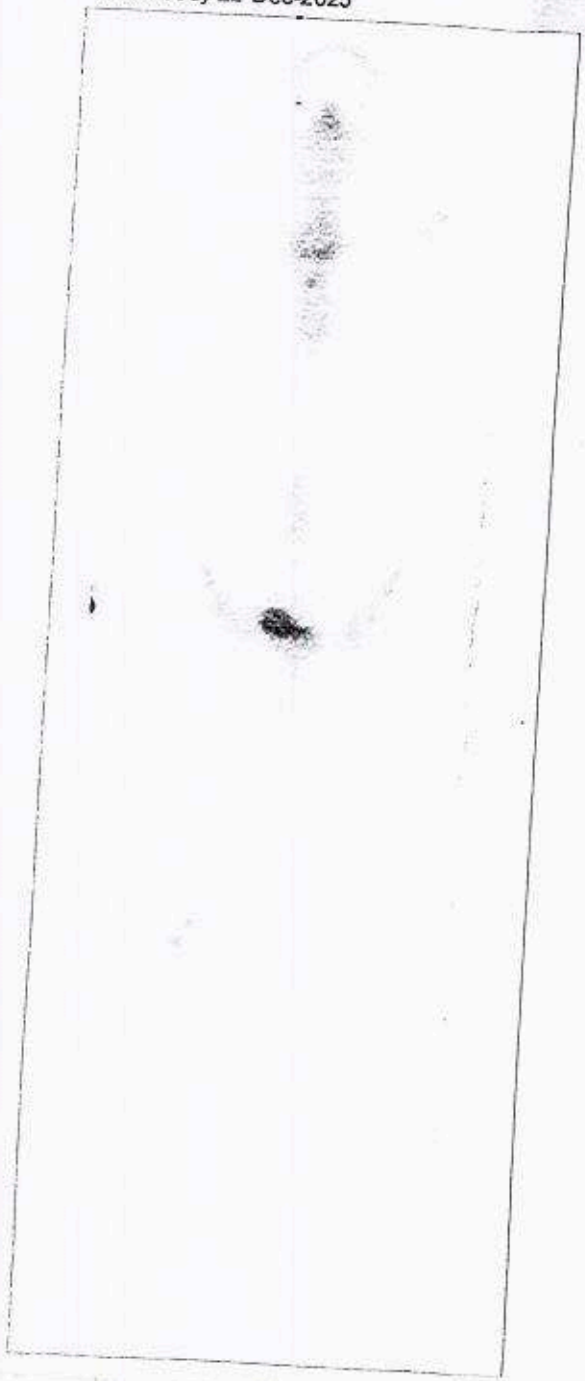
JIHAN FATTOUH ATTA ALLA DOB: 22-Dec-1981 ID: BONE SCAN SEX: F
STUDY DATE: 22-Dec-2025

**Patient Name: JIHAN FATTOUH ATTA ALLA**
**Study Name:**

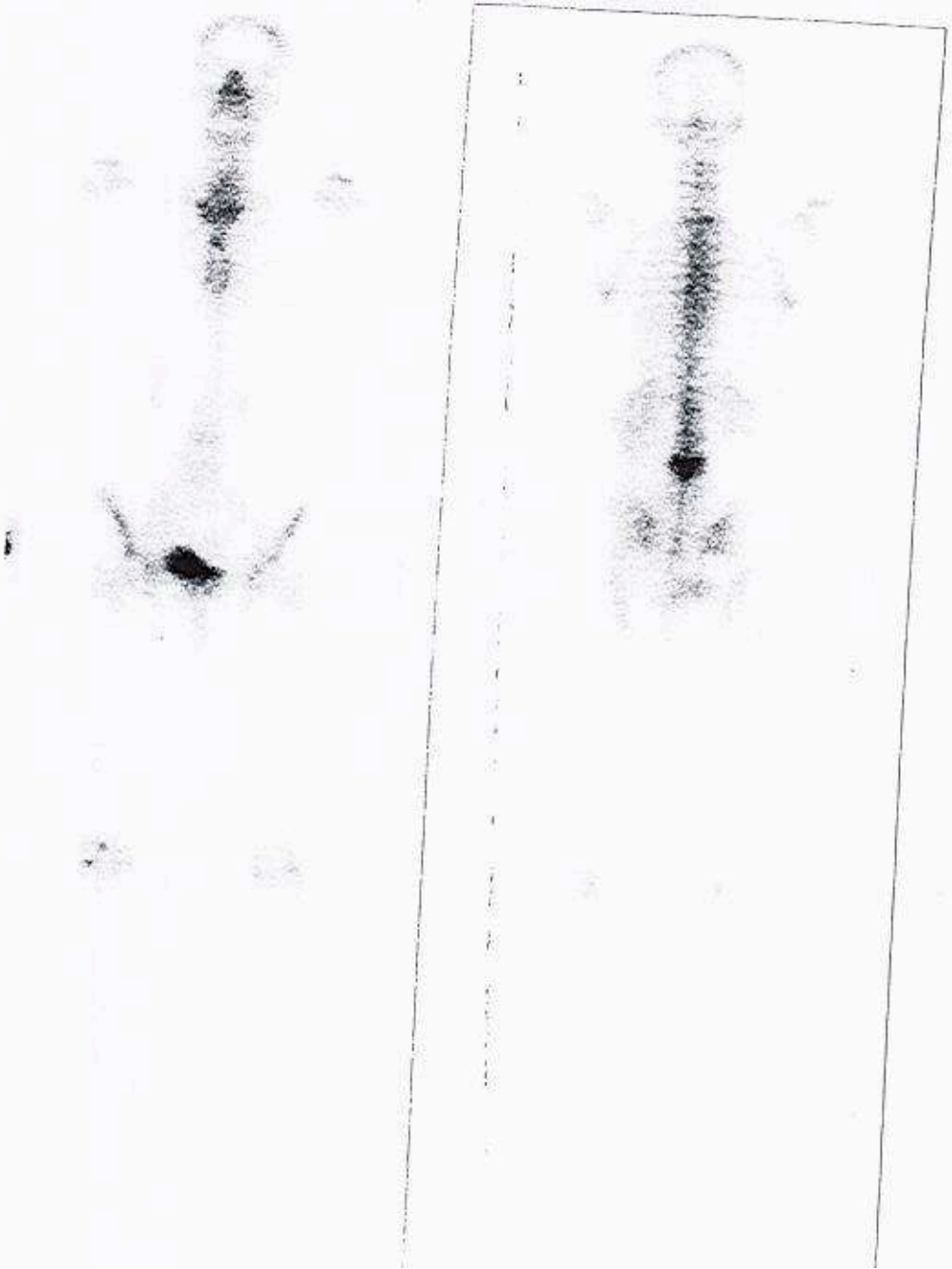
**Patient ID: BONE SCAN**
**Study Date: 22-Dec-2025**

**DOB: 22-Dec-1981**

Wholebody 22-Dec-2025



Wholebody 22-Dec-2025



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| Name : GEHAN FATOUH ATTALLAH | Date : 29-12-2025 11:48 |
| Branch : Portable | Medical No. : GRI-121957 |
| Ref.Physician Prof : | Ins.Company : |

# MRI EXAMINATION OF THE LUMBO-SACRAL SPINE

## Technique of examination:

- ☐ Sagittal T1 & T2 WIs
- ☐ Axial T1 & T2 WIs
- ☐ No IV contrast was given.

## MRI Findings:

- Straightened lumbar curve.
- Normal dimensions of the bony lumbar spinal canal.
- No vertebral structural collapse or marrow signal alteration.
- Spondylodegenerative changes of the lumbar spine manifested by:
  - Anterior marginal osteophytic lipping of the vertebral end plates
  - Diminished height and bright T2 signal intensity of the lumbar discs denoting their degeneration.
- L1-2, L2-3, L3-4 and L5-S1 diffuse posterior disc bulge is seen effacing the epidural fat, flattening the theca and encroaching on the lower aspect of the corresponding exit neural foramina.
- L4-5 posterior disc protrusion seen effacing the epidural fat, indenting the theca and encroaching upon both corresponding neural exit pathways.
- Normal MRI appearance of the conus medullaris and cauda equine nerve roots.
- No gross facetal arthropathy.
- No pre-vertebral or para-vertebral soft tissue signal abnormalities.

## Opinion:

- ***Spondylodegenerative changes with multilevel disc lesions.***

*Dr. Asser Ahmed*

*Dr. Asser Ahmed*